

THE BIRTH PLAN TEMPLATE

Know your job in the room before you're in it.

A birth plan isn't a script — it's a conversation you have before the room gets loud. What to decide together, what to tell the care team, and what to do when the plan changes anyway.

BEFORE YOU WRITE IT

A birth plan is a conversation, not a contract.

Hospitals don't run off scripts, and neither does labor. The point of a birth plan isn't controlling exactly what happens — it's making sure you and your partner have actually talked through the big decisions before you're both exhausted and getting asked to make them on the spot. Talk through the sections below together, then boil it down to one page for the care team.

Decide Together, Before You're There

The choices that are easiest to think through calmly, at home — not mid-contraction.

- ☐ **Pain management** — epidural from the start, wait and see, or unmedicated? Know her preference and the backup.
- ☐ **Who's in the room** — partner only, a doula, family in the waiting room, no visitors until she says so.
- ☐ **Delivery positions** — some hospitals default to one; ask your OB what's actually available.
- ☐ **Immediate skin-to-skin** — standard at most hospitals, but say it out loud anyway.
- ☐ **Delayed cord clamping** — a 30–90 second delay is common; ask what's routine at your hospital.
- ☐ **Who cuts the cord** — you, her, or nobody has strong feelings either way.
- ☐ **Photos or video during delivery** — decide the line before you're holding a phone in a tense moment.

Feeding & After

What happens in the first hours after birth, before you're sent home.

- ☐ **Breastfeeding, formula, or both** — the plan can change; write down the starting intention.
- ☐ **Rooming-in** — baby stays in the room with you vs. the nursery overnight.
- ☐ **Visitors after birth** — who, when, and how long. Decide it as a couple, not in the moment.

When the Plan Meets the Room

Most birth plans change at least a little. Some change completely, fast.

The goal isn't a plan that survives every scenario — it's knowing your default answers so a change in the room doesn't have to turn into a debate in the room. Walk through these together too, even though nobody wants to.

- ☐ **Emergency C-section** — who's allowed in the OR, and what happens to the rest of the plan if this is how it goes.
- ☐ **Baby needs the NICU** — who goes with the baby, who stays with her. Decide it now, not in the hallway.
- ☐ **Induction** — if labor needs a push, what order of interventions are you both comfortable starting with.
- ☐ **A change of mind on pain relief** — nobody has to stick to the original plan. Make peace with that in advance, out loud.

For the Care Team

What actually helps the nurses and doctor who walk in not knowing either of you.

- ☐ **Allergies and medical history** — hers, and anything relevant about the pregnancy so far.
- ☐ **Specific requests, ranked** — the two or three that matter most, not a wish list of twenty.
- ☐ **Previous birth history**, if this isn't the first — what worked, what didn't.
- ☐ **Pediatrician's name and info** — you'll be asked before you leave.

Print one page, not five. Hand a nurse a single-page summary and they'll actually read it. Hand them five pages during a shift change and it becomes background noise. Keep it short enough to scan in under a minute.

Your Actual Job in That Room

The plan is the easy part. This is the part nobody hands you a template for.

- ☐ **Be the calm one.** She reads your face before she reads the monitor. Steady, not silent.
- ☐ **Be the advocate.** Ask the questions she doesn't have bandwidth to ask. Speak up if something feels off.
- ☐ **Run the room.** Visitors, phone calls, the door — that's your job so it's never hers.
- ☐ **Hold the plan loosely.** The second it stops matching reality, let it go and focus on what's actually happening.

Whatever's actually happening in that room — a plan that goes exactly as written or one that falls apart in ninety seconds — that's what **the Dudela Prep Kit** walks through hour by hour: six pillars, nine printable tools, real founder stories from a NICU stay and an emergency C-section.

Real Talk from John & Mike

Neither of our birth plans survived contact with reality, and we wrote a lot of pages. John's twins arrived early and spent a month in the NICU — half the plan never got used. Mike's fully-mapped birth plan fell apart into an emergency C-section in about ninety seconds, and every preference on the page became irrelevant in the time it takes to read this sentence. Write the plan. Talk it through together. Then hold it loosely — your actual job is showing up steady for whatever the room actually needs, not the version you planned for.

— John & Mike, Dudela

This template is general guidance, not medical advice. Follow your OB or midwife's specific instructions, and your hospital's own intake and consent process, over anything written here.